

SUNRISE MULTISPECIALITY CLINIC

DISCHARGE SUMMARY

DEPARTMENT OF INTERNAL MEDICINE

Patient Details

General Information

UHID: SMC/IM/2026/1142

IP No: IP/2607/0934

Patient Identifier

Ward/Bed Number: GENERAL WARD – B14

Name: **MR. ROHAN MEHTA**

Address: Velachery, Chennai, Tamil Nadu, INDIA

Age/Sex: 53 / Male

Treating Consultants

Primary Consultant: **Dr. Ananya Rao**

Consultant: DR. ANANYA RAO, MD (INTERNAL MEDICINE)

Policy Details: Self-Paid

Date of Admission: 18/07/2026 – 08:20 AM

Date of Discharge: 21/07/2026 – 03:45 PM

Diagnosis

Chronic Kidney Disease – Stage 3a
Type 2 Diabetes Mellitus (Known Case)
Systemic Hypertension

Chief Complaints

- Swelling of both feet – 2 weeks
 - Generalized fatigue
 - Reduced urine output
-

History of Present Illness

Known case of Type 2 Diabetes Mellitus (5 months) and Hypertension. Patient presented with bilateral pedal edema and persistent fatigue. Reports reduced urine output over the last 10 days. No fever or dysuria.

Home blood sugar readings moderately controlled.

Past History

- Type 2 Diabetes Mellitus – On Metformin + Sitagliptin
 - Hypertension – On Telmisartan
 - No known drug allergies
-

Family History

Father – Type 2 Diabetes Mellitus
No known renal disease in family

Clinical Examination

General Examination

Pallor: Mild
Icterus: No
Cyanosis: No
Clubbing: No
Edema: Bilateral Pedal Edema Present

Temperature: 98.6°F
Pulse: 84 /min
BP: 154/96 mmHg
RR: 18 /min
SpO₂: 98% on Room Air

Systemic Examination

CVS: S1 S2 +
RS: NVBS +
P/A: Soft
CNS: Conscious, Oriented

Investigation Reports

Serum Creatinine: 1.8 mg/dL
Blood Urea: 48 mg/dL
eGFR: 48 mL/min/1.73m²
Urine Routine: Protein +1
Urine Albumin-Creatinine Ratio: 165 mg/g
HbA1c: 7.8%
Hemoglobin: 10.9 g/dL

Ultrasound Abdomen: Mildly increased renal cortical echogenicity

Course in the Hospital

Patient admitted for evaluation of renal dysfunction. Investigations revealed reduced eGFR consistent with Stage 3a Chronic Kidney Disease, likely secondary to diabetic nephropathy and long-standing hypertension.

Metformin dosage reviewed due to renal function. Blood pressure control optimized.

Nephrology referral advised.

Treatment Given During Hospital Stay

TAB. TELMISARTAN 40mg – 1-0-0
TAB. SITAGLIPTIN 50mg – 1-0-0
TAB. METFORMIN – Dose adjusted
TAB. ATORVASTATIN 10mg – 0-0-1
IVF – Restricted
Low salt renal diet initiated

Condition on Discharge

Vitals Stable: Yes
Edema: Reduced
Renal Function: Stable but impaired

Advice on Discharge

- Strict blood pressure control
 - Renal diet (Low salt, moderate protein restriction)
 - Daily weight monitoring
 - Avoid NSAIDs
-

Follow-up

Review after 2 weeks with RFT and electrolytes.
Nephrology consultation scheduled.

For Appointment Contact: +91 44 4000 2248

Dr. ANANYA RAO

Consultant – Internal Medicine
(Signature)

Prepared By: Medical Records Department

Sunrise Multispeciality Clinic
Chennai